
Patient: **Monson, Berdia M** Sex: Female DOB: 05/01/1937
Exam: IVR PORT PLACEMENT/INSERTION TUNNEL CV CATH W SUBQ PORT Acc# 4909FDD29A16
(36561)
DOS: 04/21/2026 07:30 AM MRN# 1095086
Ref. Phys: APPALANAIDU SASAPU

PORT PLACEMENT

CLINICAL INDICATION: 88 years Female with history of endometrial cancer. Plan for infusaport placement.

OPERATORS: Blake B. Jacks, MD

REFERRING PHYSICIAN: Appalanaidu Sasapu, MD

INFORMED CONSENT:

The patient's diagnosis, treatment plan/procedure, risks and benefits, treatment alternatives, complications, and prognosis with and without treatment were explained in plain language. Informed consent was obtained and we were asked to proceed with the procedure

A verbalized timeout was performed before the procedure with the required team present. The patient's name, date of birth, procedure, site, equipment as well as pertinent labs, medications and allergies were reviewed.

All elements of maximal sterile barrier technique were utilized during this procedure including cap, mask, sterile gown, sterile gloves, large sterile sheath, hand hygiene, and 2% chlorhexidine for cutaneous antisepsis. Sterile Ultrasound technique with sterile gel and sterile probe covers was also utilized.

Monitored anesthesia care was performed by the nurse anesthetist/anesthesiologist.

ANTIBIOTICS:

Intravenous antibiotics were administered within 1 hour of the procedure.

TECHNIQUE:

After informed consent was obtained, the patient was brought into the interventional radiology suite and placed on the angiography table in the supine position. A pre-procedure time out was performed. Ultrasound was used to identify the right internal jugular vein. The right neck and chest were prepped and draped in the usual sterile fashion. 1% lidocaine was used for local anesthetic. A small skin nick was made with an 11 blade scalpel. A 21-gauge micropuncture needle was advanced into the vein under direct ultrasound visualization. A mandril wire was then advanced centrally. The needle was exchanged for a 5-French micropuncture sheath. The inner portion of the dilator and wire were secured in place with a flow switch. Attention was then turned to the right upper chest. After making a preliminary mark with a marker, local anesthesia

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was applied superficially at the incision site and then in the deeper tissues of the pocket and tunnel. An incision was made approximately one finger breadth below the clavicle and the pocket was created. After testing that the reservoir of the port fit into the pocket, the port catheter was then tunneled antegrade to the venotomy site. The wire and inner dilator of the micropuncture sheath were removed and a 0.035 wire was advanced into the IVC. The micropuncture sheath was exchanged for a peel away sheath. The wire and inner portion of the introducer were removed. The catheter was inserted so that the tip was in the right atrium and the peel-away was removed. The catheter was cut to the appropriate length and then attached to the port reservoir. The reservoir was then placed into the pocket. The port was tested by aspirating and flushing and it was then dwelled with the appropriate heparinized solution. The pocket was closed in with deep interrupted sutures and Dermabond was applied to the incision and venotomy site. A sterile dressing was placed and the patient was returned to the recovery area for monitoring.

DISCUSSION:

The right internal jugular vein is patent and compressible. DICOM image was stored for PACS. Subsequent images demonstrate the wire within the IVC. Final spot radiograph demonstrates the catheter tip at the right atrium. The infusaport aspirated and flushed easily. The patient tolerated the procedure well with no immediate complications.

Prosthetic devices, grafts, tissues, transplants or devices implanted: 8 French Bard Vaccess port
Estimated blood loss: Minimal.

Complications: None.

Fluoroscopy time: 7 seconds with a dose area product of 0.0299 mGy.m2

IMPRESSION:

Successful placement of a right-sided infusaport, ready for immediate use.

Electronically Signed By:

Blake Jacks at 04/21/2026 14:28